AR 2EB

ALAMEDA RESEARCH
2: ELECTRIC BOOGALOO
LONG HORIZONS · STRUCTURAL SHIFTS · IMAGINATION

INTERNAL RESEARCH · MEMO · NOT INVESTMENT ADVICE · AI-ASSISTED

NovoCure

22 October 2026 · the qualitative

$15.59

NASDAQ : NVCR · Mature-Company DCF

$1.7B mkt cap · 109M sh · $1.03B cash, $560M debt · $655M revenue FY25 (+8%, ~78% GM); GAAP net loss -$136M, adj-EBITDA -$34M; ~$0.47B NET CASH; 4 FDA-approved/filed indications: Optune Gio (GBM), Lua (NSCLC, early), Pax (pancreatic), METIS (brain mets); FRESH NEGATIVE: Jun-18-2026 Phase-3 TRIDENT (newly-dx GBM) MISSED → -18%; SBC ~$100M+/yr

THE CENTRAL QUESTION

NovoCure trades at ~2.7x sales with net cash and an unproven new-indication tail — does the optionality beyond glioblastoma justify the price, or does it stay a loss-making device story?

NovoCure sells Tumor Treating Fields (TTFields), a wearable oncology device anchored by Optune Gio in glioblastoma. FY2025 revenue was $655M (+8%) at a 78% gross margin, yet it posted a $136M GAAP loss — funded by ~$0.47B net cash, not insolvency risk. At $16.53 the stock is cheap on sales; the upside lives in new FDA-approved indications (Optune Lua/lung, Pax/pancreatic, METIS) multiplying the TAM beyond GBM. Weighted fair value $14.97, ~9% below spot — modestly rich; the June 2026 TRIDENT miss caps the GBM leg.

PROBABILITY-WEIGHTED EXPECTED VALUE

most likely case (base) -10.2% · 75% at or below spot

$14.97 expected fair value today

-3.9% vs spot $15.59

FORWARD COMPOUNDED VALUE

+5y	+10y	+15y	+20y
$23.34	$36.41	$56.83	$88.76
1.50× spot	2.34× spot	3.65× spot	5.69× spot

WEIGHTING RATIONALE

Ultra Bear 14% GBM erodes post-TRIDENT; new indications never adopt; $4.20.
Bear 28% Slow adoption, thin leverage; small owner-FCF; $9.19.
Base 33% Moderate new-indication ramp; margin ~10%; $14.00 (-15%).
Bull 17% Lua and Pax adopt, the 78% margin levers to 14%; $23.58.
Ultra Bull 8% TTFields a multi-indication standard; $39.81.

Bull (17%) + Ultra Bull (8%) together contribute $7.19 — 48% of the $14.97 expected value despite 25% combined probability. Today's spot ($15.59) sits 4% above the weighted expected; the forward-weighted value reaches spot within a year.

PRICE + PROJECTIONS · HISTORICAL THROUGH PROBABILITY-WEIGHTED FORWARD



ULTRA BEAR	BEAR	BASE	BULL	ULTRA BULL
$4.20	$9.19	$14.00	$23.58	$39.81
-73.1% vs spot	-41.1% vs spot	-10.2% vs spot	+51.3% vs spot	+155.4% vs spot
CAGR +1.8% WACC 10.5% CAGR +1.8% Prob 14%	CAGR +4.0% WACC 10.0% CAGR +4.0% Prob 28%	CAGR +5.6% WACC 9.5% CAGR +5.6% Prob 33%	CAGR +8.2% WACC 9.0% CAGR +8.2% Prob 17%	CAGR +10.8% WACC 8.5% CAGR +10.8% Prob 8%
GBM erodes; new indications fail	Slow adoption, thin operating leverage	New indications ramp; margin inflects	Lua/Pax adopt; the margin levers	TTFields a multi-indication standard

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NovoCure scenario narratives

PROBABILITY WEIGHTS

Ultra Bear 14% Bear 28% Base 33% Bull 17% Ultra Bull 8% · Weighted expected $14.97 (-3.9% vs spot)

ULTRA BEAR $4.20

-73.1% vs spot

Probability 14%

GBM erodes; new indications fail

WHY 14%

Roughly one-in-seven. Requires both the GBM franchise softening after TRIDENT and the new indications stalling, while the gross margin never translates into operating profit — plausible given the long loss history, but it ignores three already-approved labels.

WHAT HAPPENS

Revenue grows only ~1.8% as the post-TRIDENT GBM base flattens and the newer labels fail to convert into real prescriptions. Active patients stall near today's ~4,800, and continuous-wear compliance keeps real-world uptake well below the approved population.

Owner-FCF margin starts at -10% and crawls to 2% — the operating leverage never arrives after a decade of losses. At ~1.8% growth and a barely-positive terminal margin the DCF is $4.20, ~75% below spot. Net cash prevents distress, but the equity is worth little if TTFields stays a single-indication niche.

BEAR $9.19

-41.1% vs spot

Probability 28%

Slow adoption, thin operating leverage

WHY 28%

About 28%. The modal disappointment path — labels exist but real-world wear, reimbursement gating, and heavy stock comp blunt the margin inflection. Consistent with a company that has grown revenue but never produced operating profit.

WHAT HAPPENS

Device adoption stays sluggish: Optune Lua is still only ~$3M a quarter, and Pax ramps slowly against entrenched chemo and radiation pathways. Revenue compounds ~4% as new indications add patients but oncologist and patient inertia caps the pace.

The owner-FCF margin moves from -5% to 7% — positive eventually, but thin, because SBC over $100M a year keeps true owner cash well below the adj-EBITDA headline. The DCF lands at $9.19, ~44% below spot: a real, funded business that simply doesn't earn enough to support the current price.

BASE $14.00

-10.2% vs spot

Probability 33%

New indications ramp; margin inflects

WHY 33%

The plurality outcome at ~33%. New indications adopt at a moderate, not heroic, rate and the long-promised operating leverage finally shows up around 10% — neither the post-TRIDENT erosion nor a platform breakout, just steady execution on approved labels.

WHAT HAPPENS

Revenue compounds ~5.6% as Optune Lua and Pax build a modest second and third leg on top of GBM, and METIS adds an NSCLC-brain-mets indication after its PMA filing. Active-patient growth continues at roughly the current single-digit pace across a widening label set.

The owner-FCF margin inflects from -2% toward 10% as the 78% gross margin starts to cover a more fixed cost base. That yields a $14.00 DCF, ~15% below spot — the price asks for more than moderate multi-indication adoption and a believable, if late, path to profitability.

BULL $23.58

+51.3% vs spot

Probability 17%

Lua/Pax adopt; the margin levers

WHY 17%

About 17%. Needs genuine commercial traction in the new indications plus the first durable operating profit in the company's history — both observable in active-patient counts and segment revenue, neither yet demonstrated at scale.

WHAT HAPPENS

Optune Lua and Pax adopt well in their approved settings, and METIS converts, so revenue compounds ~8.2%. Active patients climb materially as TTFields becomes a routine option in multiple tumor types rather than a GBM-only therapy, with reimbursement coverage broadening alongside the labels.

Operating leverage finally arrives: the owner-FCF margin runs from 1% to 14% as the high gross margin drops through a stable cost base. The DCF is $23.58, ~43% above spot. This requires the device-adoption gap (labels outrunning real prescriptions) to close, which it has not yet across the newer indications.

ULTRA BULL $39.81

+155.4% vs spot

Probability 8%

TTFields a multi-indication standard

WHY 8%

About 8%. The full platform thesis — TTFields as a multi-tumor standard of care with proven leverage. The low weight reflects unproven leverage and the fresh TRIDENT miss, not the absence of a plausible mechanism.

WHAT HAPPENS

TTFields establishes itself as a standard modality across several solid tumors, with positive trials extending the platform beyond today's labels. Revenue compounds ~10.8% as the addressable population multiplies and continuous-wear devices become a routine part of oncology protocols.

The owner-FCF margin scales from 3% to 18% as a unique, patented modality earns device-like economics on a much larger base. The DCF reaches $39.81, ~141% above spot. This is the platform outcome: the IP and clinical-evidence moat compounding across indications rather than defending a single one.

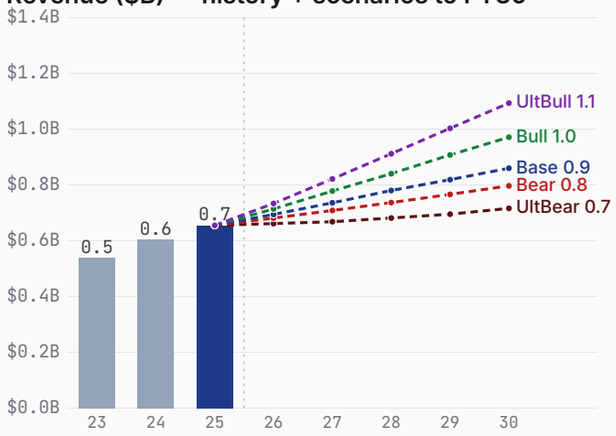
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NovoCure business snapshot

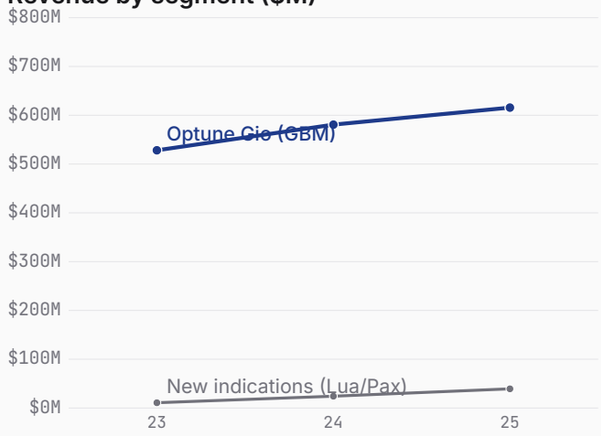
Bear $9.19 Base $14.00 Bull $23.58

FY23-FY25 history + FY26-FY30 scenario projections · fiscal years end Dec 31 · FY2025 + Q1'26 results

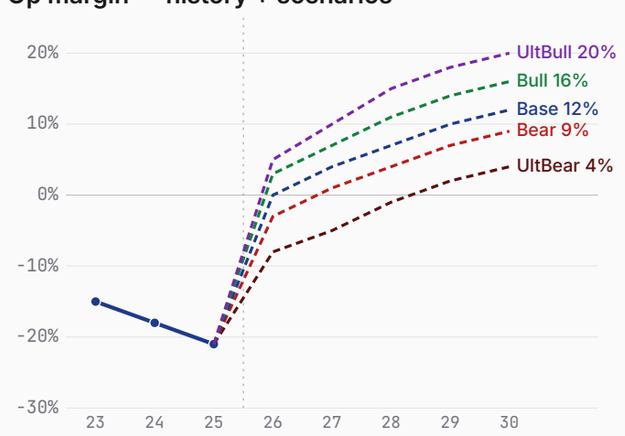
Revenue ($B) — history + scenarios to FY30



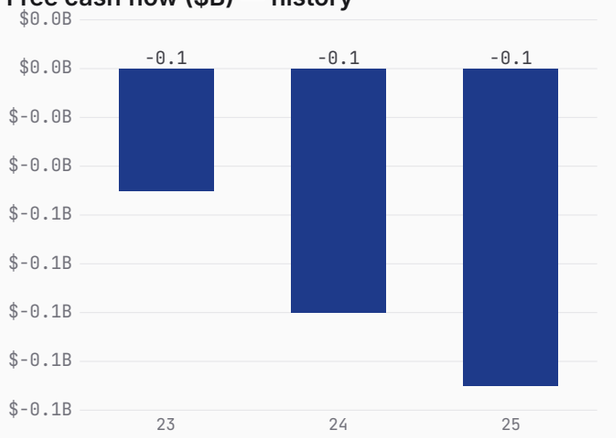
Revenue by segment ($M)



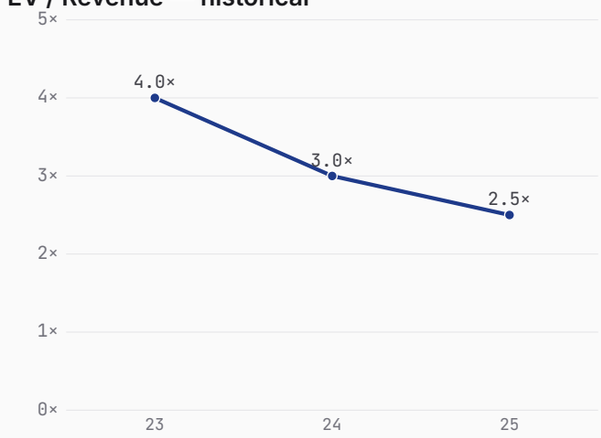
Op margin — history + scenarios



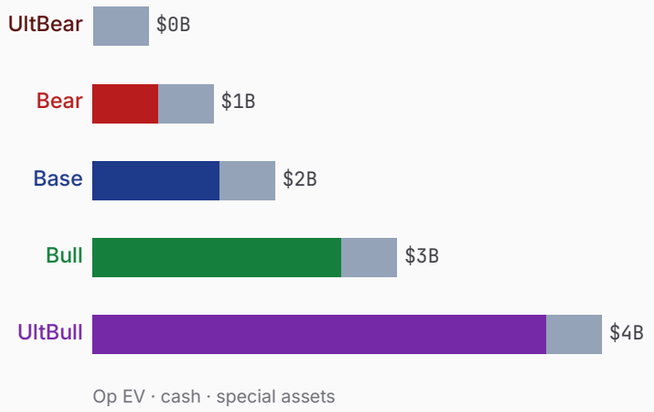
Free cash flow ($B) — history



EV / Revenue — historical



Equity build (Op EV + net cash)



Sources: NovoCure FY2025 10-K + Q1'26 results (CIK 1645113), FY26 guidance, TRIDENT/PANOVA-3/METIS readouts. Revenue as a growth-rate path off FY25; FCF = revenue x an SBC-adjusted owner-FCF margin (starts negative — device operating leverage is unproven and SBC is ~$100M+/yr); Gordon terminal at scenario WACC. EV/sales vs med-tech / oncology peers.

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NovoCure 7 Powers · the moat, scored

Bear $9.19 Base $14.00 Bull $23.58

Arena. Solid-tumor oncology, where a wearable TTFields device competes for oncologist and patient mindshare against systemic drugs and radiation, not against a rival device.

POWER AUDIT
dominant-power durability: medium

POWER AUDIT · 7 POWERS

Scale Economies	Some fixed-cost leverage in R&D and field operations, but the model is still loss-making after years.
Network Economies	No network effect; value to one patient does not rise with the number of other users.
Counter-Positioning	A non-systemic modality incumbents (pharma, radiation) cannot easily add without cannibalizing or re-tooling around drugs.
Switching Costs	Modest — an in-progress treatment course is sticky, but adoption is per-line and physician-driven, not locked-in.
Branding	Optune has oncology recognition, but prescribing follows clinical evidence and reimbursement, not brand.
Cornered Resource · thesis leans here	The TTFields IP and patented device platform are unique — no direct device competitor delivers this modality.
Process Power	Accumulated regulatory and trial know-how across indications, but reproducible by a well-funded entrant over time.

PEER SET

Standard chemo / radiation · n/a (standard of care)
The real competition for oncologist mindshare; entrenched, reimbursed, default pathway.

Pharma oncology (IO / targeted) · large-cap pharma multiples
Systemic drugs competing for the same treatment line; vastly larger R&D budgets.

Other neuro-oncology device / trials · early-stage / venture
Few direct device rivals today; the threat is future entrants replicating the modality.

Build-vs-adopt inertia · n/a
Continuous-wear compliance and physician habit, not a competitor, gate real-world uptake.

THREAT VECTORS · FALSIFIERS

Cornered Resource · Future device entrants
falsifier: A competing electric-field or wearable modality clearing trials and eroding the patent/platform exclusivity.

Counter-Positioning · Payers / reimbursement
falsifier: Coverage restrictions or denials gating new indications, which keeps real prescriptions below approved populations.

Process Power · New-indication trial risk
falsifier: A second pivotal miss after TRIDENT in lung, pancreatic, or brain mets — undercutting the clinical-evidence moat that justifies each new label.

COMPETITIVE READ

NovoCure has a genuine cornered resource — the TTFields IP and the only approved device for this modality — reinforced by a per-indication clinical-evidence moat (PANOVA-3, METIS). That makes the modality largely uncontested by direct device rivals. But the Power is medium-durability, not high: real-world adoption lags labels, reimbursement gates uptake, and the June 2026 TRIDENT miss shows the moat must be re-won trial by trial. The falsifier is a second pivotal miss or flat active-patient growth — either would prove the platform cannot extend beyond GBM.

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NovoCure show your work

Bear $9.19 Base $14.00 Bull $23.58 · Weighted $14.97 (-3.9%)

ULTRA BEAR $4.20

ASSUMPTIONS

5y revenue CAGR (%)	+1.8
Year-5 op margin (%)	4.0
WACC (%)	10.5
Terminal growth (%)	2.5
Terminal FCF ($B)	$0.01B
Starting revenue ($B)	0.66
Scenario probability (%)	14

5-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY26	0.66	-8%	-0.07	-0.06
FY27	0.67	-5%	-0.05	-0.04
FY28	0.68	-1%	-0.02	-0.01
FY29	0.70	+2%	+0.00	+0.00
FY30	0.72	+4%	+0.01	+0.01

Σ PV explicit FCF	-0.11
+ PV terminal (g 2.5%)	0.11

EQUITY BUILD · $B & PER SHARE

Operating EV	$0.00B
+ Cash & investments	$1.03B
− Total debt	$0.56B
= Equity value	$0.47B
FY30 shares (M)	112
= Expected per share	**$4.20**

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$6.92	$11.40	$18.78	$30.94
0.44×	0.73×	1.20×	1.98×

BEAR $9.19

ASSUMPTIONS

5y revenue CAGR (%)	+4.0
Year-5 op margin (%)	9.0
WACC (%)	10.0
Terminal growth (%)	3.0
Terminal FCF ($B)	$0.06B
Starting revenue ($B)	0.66
Scenario probability (%)	28

5-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY26	0.68	-3%	-0.03	-0.03
FY27	0.71	+1%	-0.01	-0.01
FY28	0.74	+4%	+0.01	+0.01
FY29	0.77	+7%	+0.04	+0.03
FY30	0.80	+9%	+0.06	+0.04

Σ PV explicit FCF	+0.04
+ PV terminal (g 3.0%)	0.51

EQUITY BUILD · $B & PER SHARE

Operating EV	$0.55B
+ Cash & investments	$1.03B
− Total debt	$0.56B
= Equity value	$1.02B
FY30 shares (M)	111
= Expected per share	**$9.19**

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$14.80	$23.84	$38.39	$61.83
0.95×	1.53×	2.46×	3.97×

BASE $14.00

ASSUMPTIONS

5y revenue CAGR (%)	+5.6
Year-5 op margin (%)	12.0
WACC (%)	9.5
Terminal growth (%)	3.5
Terminal FCF ($B)	$0.09B
Starting revenue ($B)	0.66
Scenario probability (%)	33

5-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY26	0.69	+0%	-0.01	-0.01
FY27	0.74	+4%	+0.01	+0.01
FY28	0.78	+7%	+0.04	+0.03
FY29	0.82	+10%	+0.07	+0.05
FY30	0.86	+12%	+0.09	+0.06

Σ PV explicit FCF	+0.13
+ PV terminal (g 3.5%)	0.94

EQUITY BUILD · $B & PER SHARE

Operating EV	$1.07B
+ Cash & investments	$1.03B
− Total debt	$0.56B
= Equity value	$1.54B
FY30 shares (M)	110
= Expected per share	**$14.00**

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$22.04	$34.70	$54.62	$85.98
1.41×	2.23×	3.50×	5.52×

BULL $23.58

ASSUMPTIONS

5y revenue CAGR (%)	+8.2
Year-5 op margin (%)	16.0
WACC (%)	9.0
Terminal growth (%)	4.0
Terminal FCF ($B)	$0.14B
Starting revenue ($B)	0.66
Scenario probability (%)	17

5-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY26	0.71	+3%	+0.01	+0.01
FY27	0.78	+7%	+0.04	+0.03
FY28	0.84	+11%	+0.08	+0.06
FY29	0.91	+14%	+0.11	+0.08
FY30	0.97	+16%	+0.14	+0.09

Σ PV explicit FCF	+0.26
+ PV terminal (g 4.0%)	1.84

EQUITY BUILD · $B & PER SHARE

Operating EV	$2.10B
+ Cash & investments	$1.03B
− Total debt	$0.56B
= Equity value	$2.57B
FY30 shares (M)	109
= Expected per share	**$23.58**

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$36.28	$55.82	$85.89	$132.15
2.33×	3.58×	5.51×	8.48×

ULTRA BULL $39.81

ASSUMPTIONS

5y revenue CAGR (%)	+10.8
Year-5 op margin (%)	20.0
WACC (%)	8.5
Terminal growth (%)	4.5
Terminal FCF ($B)	$0.20B
Starting revenue ($B)	0.66
Scenario probability (%)	8

5-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY26	0.73	+5%	+0.02	+0.02
FY27	0.82	+10%	+0.07	+0.06
FY28	0.91	+15%	+0.12	+0.09
FY29	1.00	+18%	+0.16	+0.12
FY30	1.09	+20%	+0.20	+0.13

Σ PV explicit FCF	+0.41
+ PV terminal (g 4.5%)	3.42

EQUITY BUILD · $B & PER SHARE

Operating EV	$3.83B
+ Cash & investments	$1.03B
− Total debt	$0.56B
= Equity value	$4.30B
FY30 shares (M)	108
= Expected per share	**$39.81**

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$59.86	$90.01	$135.34	$203.51
3.84×	5.77×	8.68×	13.05×

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NovoCure People · Opportunity · Context · Deal

Bear $9.19 Base $14.00 Bull $23.58

Underwrite the position the way a venture investor underwrites a round — across all four legs, public or private. **People** is the leg scored here (observable only); Opportunity, Context and Deal cross-reference the rest of the memo.

PEOPLE · WHO RUNS / OWNS / FUNDS IT

Frank Leonard — 0 yrs in seat

22.7%
insider ownership

MEDIUM
key-person risk

Capital allocation (realized)
Frank Leonard became CEO on December 1, 2025 — a 15-year NovoCure veteran (prior President/COO/CFO) who replaced Ashley Cordova after her sub-one-year tenure (CEO Jan-Nov 2025), itself following founder-era CEO Asaf Danziger (2002-2024). Capital funds the TTFields oncology-device franchise and new-indication trials (~78% gross margin, net cash); no dividend or buyback. The fresh Phase-3 TRIDENT GBM miss (June 18, 2026 — overall survival 17.7 vs 17.5 months, primary endpoint not met) is the key recent operating event.

Incentive alignment
New CEO Leonard is on a CHF 750,000 base with a 90%-of-base target bonus and standard change-of-control terms. The dominant ownership figure is Executive Chairman William Doyle's ~22.7% beneficial stake (largely via WFD Ventures Fund II, over which he holds sole voting power) — concentration at the non-independent chair, not the CEO. Recent insider Form 4 activity has been routine selling.

GOVERNANCE FLAGS

- single class of ordinary shares, one vote per share (Jersey-incorporated); no controlling shareholder — largest holder Soleus ~9.1%
- non-independent Executive Chairman (Doyle, since 2016) holding ~22.7% beneficially via WFD Ventures Fund II — concentration at the chair
- CEO instability: two transitions in ~13 months (Danziger → Cordova Jan 2025 → Leonard Dec 2025)
- 8 of 11 directors independent (~73%), board declassified (annual election), with a separate lead independent director (Madden, since Feb 2026)
- founder Yoram Palti is off the board since 2018 (now CTO/consultant); his consulting fee is no longer a disclosed related-party item

THE FOUR LEGS

People ●●●○○
observable composite · 1–5

Opportunity
The scenario distribution · the business snapshot · the Arthur Indicator

Context
The 7 Powers analysis (Power Audit)

Deal
Open-market common — entry price weighed against the probability-weighted fair value + position sizing.
-3.9% vs spot $15.59

> **PEOPLE READ**
> Clean single-class one-vote shares, a ~73%-independent declassified board and a separate lead independent director are the positives. Holding the score at 3 are a non-independent Executive Chairman with a ~22.7% concentrated stake and back-to-back CEO turnover (two changes in ~13 months) — though the new CEO is a 15-year insider and the device franchise is institutionalized, so key-person risk is moderate. The June 2026 TRIDENT miss is an operating, not a governance, event.

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NovoCure supporting analysis · disclaimers · glossary

Bear $9.19 Base $14.00 Bull $23.58

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

1 **Cheap with a bounded floor**
At ~2.7x sales, net cash, 78% GM — the downside is capped, no insolvency risk even in the bear.

2 **Multiple shots on goal**
GBM, lung, pancreatic, brain-mets are four approved/filed indications; one real ramp re-rates the thesis.

3 **First pancreatic therapy in ~30 years**
PANOVA-3 made Optune Pax the first new locally-advanced pancreatic therapy in ~30 years — a differentiated position.

4 **Unproven operating leverage**
The bull needs the first durable operating profit in company history; a decade of losses and SBC over $100M a year mean that leverage is asserted, not demonstrated.

5 **Adoption lags labels**
Continuous-wear compliance keeps real prescriptions below approved populations — Optune Lua is still only ~$3M a quarter despite full FDA approval.

FALSIFICATION TRIGGERS

Bull validation
Active patients accelerating above ~4,800; Lua/Pax revenue ramping to tens of millions; owner-FCF turning durably positive.

Bear validation
Active-patient growth flattening; Lua/Pax stuck in single-digit millions; adj-EBITDA staying negative; reimbursement gating.

Reframe needed
A second pivotal trial missing (after TRIDENT) in a new indication, or GBM revenue declining — collapses the multi-indication TAM-expansion thesis.

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Scenarios, valuations, projections, and any forward-looking statements involve substantial assumptions and uncertainty. Actual results may differ materially from those projected. Past performance is not indicative of future results. The model is a model; reality is not.

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GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

Tumor Treating Fields (TTFields) — Low-intensity alternating electric fields, via a wearable device, that disrupt cancer-cell division.

Optune Gio / Lua / Pax — The approved products: Gio (glioblastoma, the base), Lua (metastatic NSCLC, early ramp), Pax (locally-advanced pancreatic, newly approved).

Active patients — The key device KPI — patients currently on therapy (~4,791 in Q1 2026); revenue scales with this count, not with label approvals alone.

TRIDENT / PANOVA-3 — Phase 3 trials — TRIDENT (newly-diagnosed GBM + radiation) MISSED its endpoint June 2026; PANOVA-3 (pancreatic) succeeded and enabled Optune Pax.

adj-EBITDA vs owner-FCF — Adjusted EBITDA (-$34M FY25) excludes stock comp; owner-FCF subtracts the ~$100M+/yr of SBC, so true owner cash starts well below the headline.

SBC — Stock-based compensation — a real, dilutive cost (a $43M charge hit Q1'26 on the Optune Pax approval) that the adj-EBITDA figure adds back.

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